

Methodist Hospital Stone Oak
Imaging Department
1139 East Sonterra Blvd
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PHONE #: 210-638-2901
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NAME: CLAYBORNE, KAREN
PHYS: Boylston, Michelle R DO
DOB: 01/11/1966 AGE: 60 SEX: F
ACCT: K509976683 LOC: K.327 1
EXAM DATE: 05/24/2026 ST: ADM IN
UNIT NO: K00395352
EXAM COMPLETE: 0808

EXAMS:

001136951 CT Ches/Abd/Pelv wo/w Con

CT CHEST ABDOMEN PELVIS WITHOUT AND WITH IV CONTRAST

DATE: 5/25/2026

HISTORY: Reason for Exam? Cancer staging DX: DYSARTHRIA Comment? Per
NSGY and Onc (K509976683)

TECHNIQUE: CT of the chest, abdomen and pelvis performed without then with
intravenous contrast.

COMPARISON: 9/25/2024 CT

FINDINGS:

CHEST:

Post treatment changes at the right lung, with mild right chest volume loss. Prior CT dated 9/25/2024 showed an extremely large right lung mass, and there is no convincing recurrent suspicious mass in the lung. Lower lung architectural distortion and probable rounded atelectasis. There is an unchanged 4 mm left upper lobe nodule, image 24. Central airways are patent. Small loculated right pleural effusion with smooth pleural thickening. No suspicious nodal enlargement in the chest. Heart size is normal. No pericardial effusion. No coronary artery calcifications. The thoracic aorta is normal in caliber.

ABDOMEN/PELVIS:

Unchanged 1 cm hyperattenuating right liver nodule on series 2 image 116, almost certainly benign. No residual viable metastasis in segment IV. No new suspicious liver lesion or biliary ductal dilatation. Gallbladder and bile ducts appear normal. Gallbladder is collapsed, with tiny stones suspected. Unremarkable spleen, pancreas, and adrenal glands. No suspicious kidney lesion or hydronephrosis.

Abdominal aorta is normal in caliber, without dissection. No ascites. There is no suspicious abdominal or pelvic lymphadenopathy.

No bowel obstruction. There is no suspicious bowel wall thickening. Appendix is normal. Mild distal colon diverticulosis without acute diverticulitis.

Bladder is mildly filled. There is an approximately 6 cm uterine fibroid.

OSSEOUS STRUCTURES: No new suspicious bone lesion. Mild sclerosis within the T3 and T9 vertebral bodies, where prior lytic metastases were noted, consistent with treated disease.

IMPRESSION:

1. Post treatment changes in the right lung, without convincing residual or recurrent disease.
2. No active metastatic disease.

Signed on 5/25/2026 9:25 AM by Travis D Lyons, M.D.

** Electronically Signed by Travis D Lyons MD on 05/25/2026 at 0925 **
Reported and signed by: Travis D Lyons, MD

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Signed Report

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CC:

TECHNOLOGIST: Smith, Tawny C

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